M E M O R A N D U M

TO: Interested parties

FROM: Jason Park

DATE: October 11, 2026

SUBJECT: Effects of the ACA Medicaid expansion on low-income adults' coverage, employment and "job lock," and implications for the Medicaid work requirements due by January 2027

EXECUTIVE SUMMARY

The Affordable Care Act's Medicaid expansion made adults with income up to 138 percent of the federal poverty level (FPL) eligible for coverage that does not depend on a job. If employer coverage "locks" low-income workers into jobs, expansion should have raised job switching, self-employment and voluntary part-time work, and it might have reduced work. This analysis uses the Census Bureau's CPS Annual Social and Economic Supplement (reference years 2008–2024, 40 states) and a modern staggered difference-in-differences estimator. **Expansion raised Medicaid coverage among low-income adults by about 13 percentage points and cut the uninsured rate by about 10 points. It did not reduce employment or hours, and it did not detectably change job mobility, self-employment or voluntary part-time work.** A conventional specification would have understated the coverage gain by a quarter to more than a half and reported a spurious 3-point rise in employment, because of how the survey measures coverage in the year just before expansion.

KEY FINDINGS

- **Large, robust coverage gains.** In the 15 states that expanded in 2014, Medicaid coverage among adults aged 26–64 at or below 138 percent of FPL rose by 13.3 percentage points (95 percent CI 10.3 to 16.3) and the uninsured rate fell by 9.9 points (CI −12.6 to −7.3), averaged over the first five years. Medicaid estimates from 11.9 to 15.4 points are obtained with alternative age ranges, timing definitions, sample windows and control groups; only the conventional comparison year gives a smaller effect.
- **Some substitution away from private coverage.** Private plans held in the person's own name fell by 3.3 points. Part of this reflects marketplace subsidies available to people at 100–138 percent of FPL in non-expansion states.
- **No reduction in work.** The share who worked during the year changed by −0.5 points (CI −1.9 to +1.0) and annual hours by −2 hours (CI −34 to +30), ruling out employment losses larger than about 0.15 per new Medicaid enrollee. Measured against the average of all pre-expansion years the employment estimate is +2.0 points, so a small increase cannot be excluded.
- **No evidence of job-lock release.** Job switching (two or more employers in a year), self-employment and voluntary part-time work did not rise. The statistically significant estimates for part-time work and multiple employers point in the opposite direction from the theory, begin before expansion and disappear in some sample windows; they are not attributable to the policy.

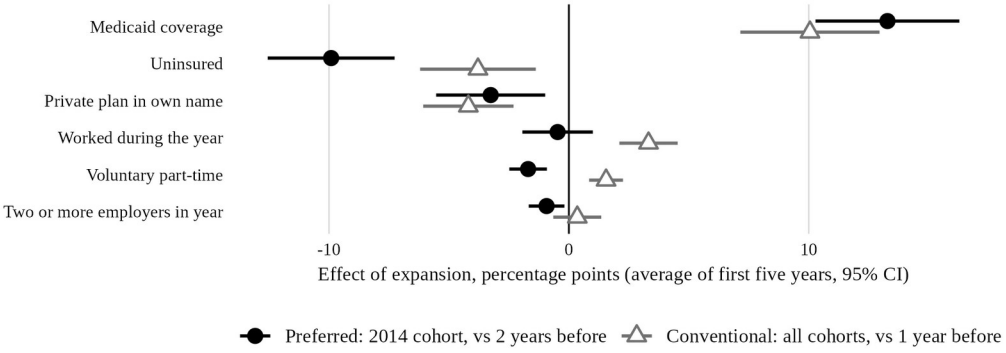


Figure. Effects of expansion, average of the first five years, with 95 percent confidence intervals.

ECONOMETRIC LIMITATIONS

- **Few clusters.** Policy varies by state: 25 states in the preferred comparison and 40 overall. Late-adopting cohorts contain one to three states each. A placebo test using those states produced spurious "effects," so findings rest on the 2014 cohort, which passed its placebo test.
- **Parallel trends are probed, not proven.** Once the contaminated year is excluded, coverage outcomes show no pre-existing linear trend, although joint tests flag year-to-year noise for Medicaid and private own-name coverage. Self-employment and employer coverage do show pre-existing trends and are not interpreted.
- **Measurement.** The survey asks in February–April about the prior calendar year and, since 2014, uses current coverage to aid recall. The year before a January expansion is therefore partly treated, so effects are measured against two years before expansion. Job mobility is proxied by having two or more employers in a year; the data do not observe job-to-job moves directly.
- **Sample definition.** The income-based sample can shift if expansion changes earnings. An education-based alternative gave internally inconsistent results and is not relied upon.

POLICY IMPLICATIONS

- **Work requirements.** Public Law 119-21 requires expansion adults aged 19–64 to document 80 hours a month of work or qualifying activity; under the Centers for Medicare & Medicaid Services' June 2026 interim final rule, states must generally implement by January 1, 2027. Because gaining Medicaid did not reduce work, conditioning Medicaid on work is unlikely to raise employment much; its main effect is likely to be on coverage. That is consistent with Arkansas's 2018 requirement, which reduced coverage without detectable employment gains (Sommers et al., 2019), and with the Congressional Budget Office's projection that the requirement will add about 5.3 million uninsured people by 2034. Implementation choices that limit procedural disenrollment of people who already work or qualify for exemptions (automated data matching, ex parte verification and simple reporting) will strongly influence the size of the coverage loss.
- **Job lock.** Relatively few adults below 138 percent of FPL hold employer coverage in their own name, so public coverage does little to change their job mobility. Job-lock arguments for decoupling insurance from employment apply mainly to higher-income workers.
- **Analytic practice.** Agencies using the CPS-ASEC to evaluate January policy changes in coverage should not use the immediately preceding year as the sole comparison. Comparing with an earlier year or the pre-period average, and reporting the year-before estimate, is a simple safeguard.

Data and methods. IPUMS CPS-ASEC survey years 2009–2025; adults 26–64 with family income at or below 138 percent of FPL; 30 expansion states (cohorts 2014–2024) and 10 non-expansion states, excluding early expanders and states with broad pre-ACA adult coverage; 680 state-year cells. Callaway and Sant'Anna (2021) estimator with never-treated controls, state-clustered standard errors, and checks against two-way fixed effects, a Goodman-Bacon decomposition, placebo dates and alternative samples. Full working paper and replication code accompany this memo.